BRIGHTWATER HEALTH PLAN

Evidence Guide: Failed Medication

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1. What this guide covers

This guide covers evidence that medicines tried for the condition did not help or could not be tolerated: pain medicines, anti-inflammatories, neuropathic medicines, weight-loss medicines and others. It is used by the Intake Coordinator who opens a review and the UM Nurse Reviewer who decides it, from the documents sent with a request to the recommendation, and by Provider Services Representatives answering the requesting office.

It comes in family physician and specialist letters for every service, and most often for spinal imaging and knee replacement.

Physiotherapy, exercise and injections are conservative treatment, and the Conservative Treatment guide covers them. A failed medicine shows that treatment was tried; it does not give the length of treatment a policy counts.

Whether a criterion is met is a separate question, settled by the questionnaire and the service's own medical policy. This guide is about the evidence.

2. How the evidence shows up

- a medicine tried without benefit or with little effect
- a medicine stopped because of side effects such as indigestion, drowsiness or dizziness
- the maximum dose reached without relief
- one medicine switched for another
- a weight-loss medicine that did not lead to lasting weight loss

3. What the reviewer checks

These are the checks the reviewer makes, in this order:

1 Name each medicine and what happened: no benefit, a side effect, or the maximum dose reached.

2 Check each medicine was for the condition of the request.

3 Never turn a list of medicines in a letter into weeks of treatment; the questionnaire decides.

4 Note a medicine still taken, and whether it helps at all.

5 Gather the failed medicines into the review for a Physician Reviewer.

4. Criteria it bears on

No criterion of the five medical policies is decided by failed medication. It supports the conservative treatment criterion of the spinal MRI and knee replacement policies, and the weight-management programme criterion of the bariatric surgery policy, without changing either.

5. When the documents disagree

The questionnaire asks no question about medicines, so a failed medicine never disagrees with it. When a letter speaks of a longer course of treatment than the questionnaire does, the Conservative Treatment guide applies.

6. What to tell the requesting office

Tell the requesting office to describe each medicine tried in the clinic letter, with what happened and for how long it was taken, and that a list of medicines never replaces the length of conservative treatment or of a weight-management programme on the questionnaire.

7. When to refer to a physician

Refer to a Physician Reviewer when the medicines are the only treatment described and the policy's treatment criterion is not met, so that the physician can weigh them.

Referring does not decide the request: a Physician Reviewer decides it, as the Utilisation Management Policy describes.

8. Recording the evidence

Each sentence's evidence is recorded on the review: the sentence, its document, the kind of evidence, the criterion it bears on and any disagreement flagged.

The review recommends one of the decisions open, and a UM Nurse Reviewer who did not open it decides, as the Utilisation Management Policy describes. Evidence informs the decision; the questionnaire and the medical policy decide the criteria.